

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: RSanchezPeriod: September 17 to October 17, 2024Report Date: October 18, 2024Coverage: Bed: 605/730h (82.9%)Chair: 522/730h (71.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

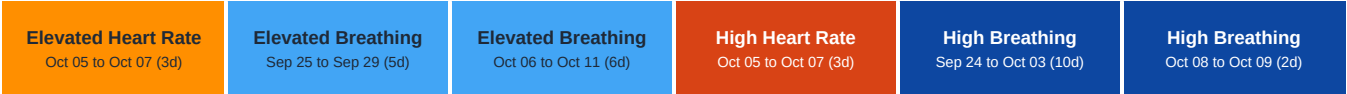
Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Patient clinical status over 31 days from Sep 17 to Oct 17

HR Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	57 bpm	50 bpm	85 bpm
Breathing	16 brpm	10 brpm	46 brpm

Last 24 hours: one elevated breathing episode in the morning, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Oct 17	9 AM to 10 AM	1h	1	Elevated Breathing	28	13	46	Check SpO2 and lung sounds; assess for fluid overload or early infection

43 episodic events Detected, spanning 50h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Very High Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
43	50h	High Breathing	1	100%

Major Findings: Sustained high breathing (avg 35, peak 58)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Sep 24	1 PM to 2 PM	15h	12	High Breathing	35	12	58	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Oct 05	2 AM to 3 AM	3h	2	High Heart Rate	104	87	115	Check temp, hydration, pain; review for arrhythmia or infection
Oct 08	8 AM to 9 AM	3h	3	High Breathing	35	12	54	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Sep 25	7 AM to 8 AM	6h	6	Elevated Breathing	26	14	57	Check SpO2 and lung sounds; assess for fluid overload or early infection
Oct 05	1 AM to 2 AM	2h	2	Elevated Heart Rate	98	57	108	Check temp, hydration, pain; review for arrhythmia or infection
Oct 06	1 PM to 2 PM	9h	8	Elevated Breathing	26	13	46	Check SpO2 and lung sounds; assess for fluid overload or early infection
Oct 08	7 AM to 8 AM	2h	2	Very High Breathing (brief)	46	14	54	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

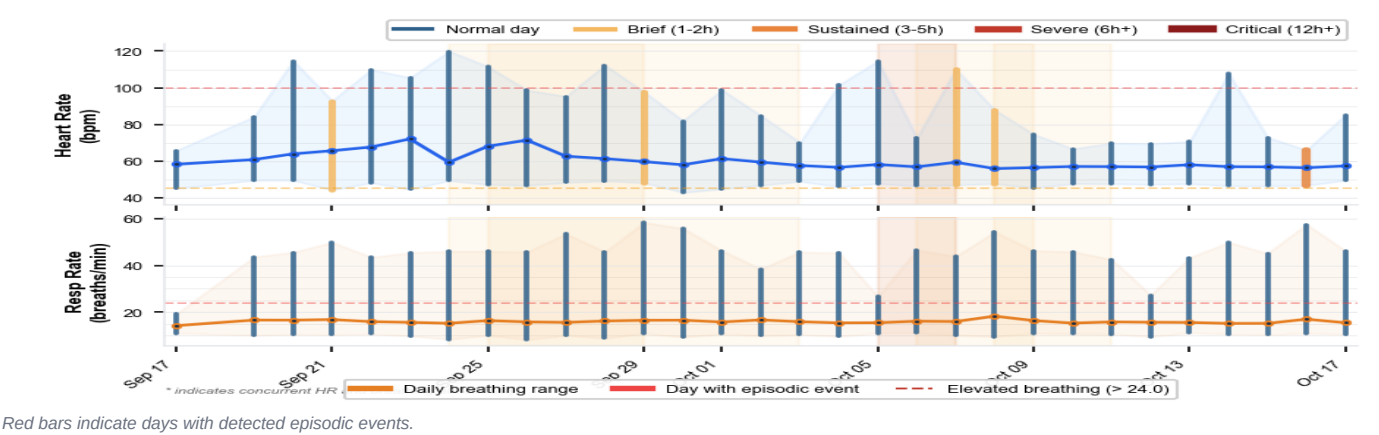
The P5 to P95 heart rate spread was 33 bpm (52 to 85), indicating significant cardiac variability.

Suggested Clinical Review Actions

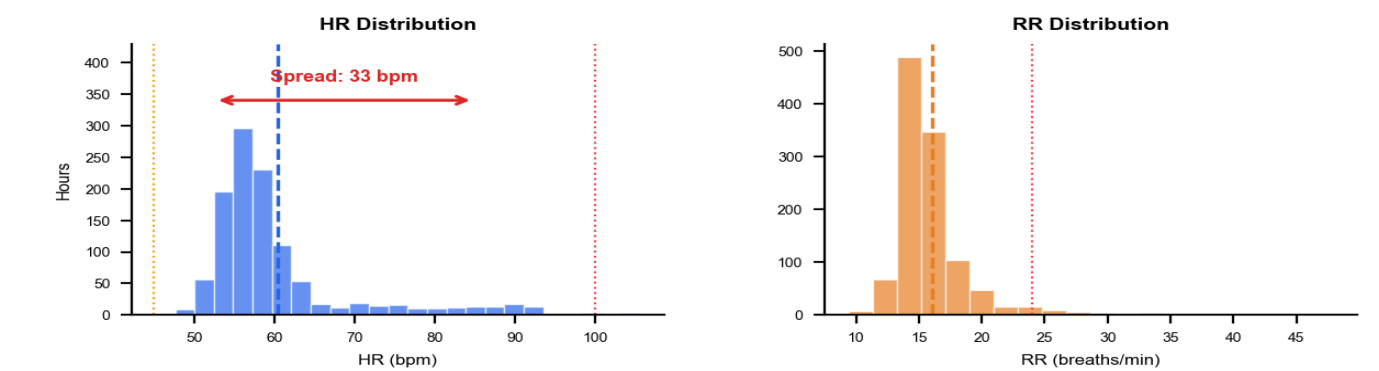
- High Breathing (avg 35 brpm, peak 58 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Elevated Breathing (avg 26 brpm, peak 46 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 104 bpm, peak 115 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Heart Rate (avg 98 bpm, peak 108 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Very High Breathing (avg 46 brpm, peak 54 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

Clinical Pattern Observations:

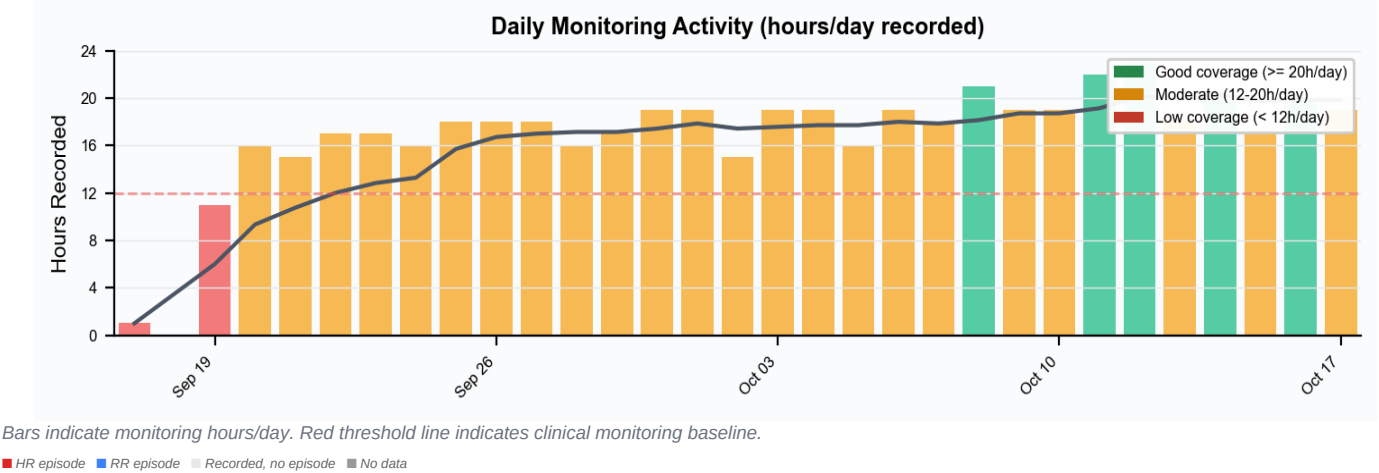
- 1. **High variability:** HR spread 32 bpm (52 to 85).
- 2. **Clustered pattern:** Episodic events on 5 of 31 days (16%).
- 3. **Daytime pattern:** Episodes concentrated during waking hours.



Vital Sign Distribution



Monitoring Activity



Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.